



Clinical Practice Guideline

Pressure Injury Prevention and Management

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Staff

Introduction

The Sir Charles Gairdner Osborne Park Health Care Group (SCGOPHCG) provides an evidence-based approach to the prevention and treatment of pressure injuries in accordance with the 'Australian Commission on Safety and Quality in Healthcare, National Safety and Quality Health Service Standards – Comprehensive Care' and the 'Prevention and Treatment of Pressure Ulcers/Injuries: Clinical Practice International Guideline, Edition 3 (2019)'.

The purpose of this policy and procedure is to establish the minimum standards for the screening, assessment prevention and management pressure injuries across the group.

Risk Statement

Non-compliance with this guideline will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Misconduct	☐
Breach SCGH Mission & Values	☐	Staff Safety	☐
Other:	☐		

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Introduction

A formal pressure injury risk screening, skin assessment and nutritional assessment will be conducted on all patients **as soon as possible**¹ and **within 8 hours of admission**.

Appropriate interventions will be initiated and documented on the care plan, outpatient notes or approved area specific assessment form for any patients identified “at risk” of, or with an existing pressure injury.

Ongoing risk assessments shall be conducted according to patient risk.

Datix Clinical Incident Monitoring System (CIMS) must be completed for:

- All hospital acquired pressure injuries
- Any existing pressure injuries that are present on admission and have deteriorated to the next stage

Definitions

Pressure Injury (PI)	Localised damage to the skin and/or underlying tissue as a result of pressure or pressure in combination with shear. Pressure Injuries, usually occur over a bony prominence but may also be related to a medical device or other object. ¹
Datix CIMS	Clinical Incident Management system used to notify clinical incidents at SCGOPHCG.
Theatre Management System (TMS)	Clinical information system which captures data about a surgical/procedural episode.
Notifications and Clinical Summaries (NaCS)	Web-based application for managing patient discharge summary information and prescriptions.

Procedure

1.0 Risk Screening and Assessment

Staff will consider the following risk factors when assessing patient risk of pressure injury ¹:

- Skin status (e.g. quality of skin, dry, mottled, presence stage I likely to develop to stage II PI)
- Body temperature (elevated)
- Perfusion, circulation and oxygenation factors
- Nutrition status
- Sensory perception limitations
- Activity and mobility limitations
- Moisture
- Older age
- Blood markers (e.g. low serum albumin levels, low haemoglobin, elevated C-reactive protein)
- General and mental health status

Additional risk factors for specific populations¹:

- Critically ill patients
- Patients with spinal cord injury
- Patients receiving palliative care
- Patients with obesity
- Patients in operating theatre

'At risk' patients are to be "flagged" using the iCM clinical handover, at bedside clinical handover and identified on the journey board, by electronic symbol or using the pressure injury risk magnet (see Appendix 5).

Staff will ensure relevant communication and referral to multidisciplinary health care staff such as:

- Wound Management Service
- Medical Officer
- Occupational Therapist
- Podiatrist
- Dietitian
- Physiotherapist

1.1 Initial Assessment

Admitted Areas

All patients must be screened and assessed for pressure injury **as soon as possible¹** and **within 8 hours of admission** by medical and/ or nursing staff using both clinical judgement and the Waterlow Risk Assessment Tool (see Appendix 3) as incorporated within the Nursing Admission Assessment Form (NAAF)

Any patient with impaired activity, mobility or sensory perception must be considered at risk.

Embedded within this tool is the Malnutrition Screening Tool (MST) used to screen for malnutrition.

A comprehensive head-to-toe skin assessment must be conducted with attention to:

- Skin over bony prominences including heels, sacrum, ischial tuberosities, femoral trochanters, elbows, malleoli, spinous processes, occiput, and ears;
- Around and under medical and therapeutic devices (such as non-invasive ventilation masks, endotracheal tubes, oxygen tubing, splints or casts, and neck immobilisation collars);
- Presence of persistent erythema or blisters;
- Localised pain especially at pressure points;
- Signs of maceration, paying attention to skin folds especially for patients with obesity;
- Localised heat, localised oedema and induration; and
- Purplish / bluish discolouration in those patients with darkly pigmented skin.

Appropriate interventions will be initiated and documented on the comprehensive care plan, clinical pathway, outpatient notes or approved area specific assessment form for any patients identified "at risk" of, or with an existing pressure injury.

Emergency Department (ED)

Adult patients will have a Waterlow Risk Assessment Tool and full skin assessment completed with the assessment findings documented in the Emergency Department Nursing Assessment Record MR858 and medical ED records.

Day Procedure Areas

In day procedure areas across SCGOPHCG, where patients will be admitted for >2hours, patients' skin integrity and pressure injury risk are assessed, and interventions implemented as indicated.

Day Procedure areas include:

- Short Stay Unit
- Day Admission Surgical Units
- Cancer Centre

- Home Link
- Dialysis
- Gastroenterology Unit
- Pain Management
- IV Therapy Lounge/Plastic Dressing Clinic
- Interventional Radiology

1.2 Reassessment

Risk assessment (clinical assessment and the Waterlow Risk Assessment Tool) will be conducted:

- After any change in medical condition, mobility or identified risk factors (e.g. cardiovascular changes, change in cognitive function etc).
- Post-surgery/procedure
- On permanent transfer to another ward
- If a pressure injury is detected
- Post fall
- Prior to discharge/transfer to another facility
- Daily if assessed as at risk of a pressure injury
- Twice weekly if generally not at risk

Comprehensive skin assessments should be conducted daily if at risk.

Patients within the Intensive Care Unit will be screened daily.

2.0 Interventions for Prevention and Treatment of Pressure Injuries

Interventions must be documented in the medical record and comprehensive care plan and document the effect of interventions implemented.

All assessment, management and interventions must be documented, signed and dated in the NAAF.

2.1 Prevention and Intervention Strategies

Staff will develop an individualised pressure injury prevention plan based on the risk assessment. The plan will consider:

Skin hygiene¹

- Keep the skin clean and appropriately hydrated
- Cleanse the skin promptly after incontinence
- Avoid the use of alkaline soaps and cleansers
- Protect the skin from moisture, consider use of barrier products
- Check all skin folds including underneath the pannus in obese patients
- Avoid vigorous rubbing of skin

Continence management¹

- Use high absorbency continence products to protect skin if patient incontinent
- Review continence products/aids regularly
- Refer to continence service as required
- Regularly toileting

Prophylactic dressing (soft silicone multi-layered foam)³

- Apply to the sacrum, heel and / or bony prominences for patients at risk or if undergoing a surgical procedure > 2 hours
- Inspect skin daily underneath dressing. If the dressing becomes soiled (e.g. due to incontinence) it must be removed and replaced.

Nutritional status¹

- Follow malnutrition action plan for “at risk” patients
- Refer patients identified as at risk of malnutrition to a Dietitian for comprehensive assessment
- Assess, and monitor nutrition and hydration status, encourage/assist intake as required.

Repositioning and early mobilisation¹

- Determine repositioning frequency with consideration to the patients:
 - Skin and tissue tolerance
 - General medical condition
 - Overall treatment objectives
 - Comfort and pain
- Reposition the individual in such a way that optimal offloading of all bony prominences and maximum redistribution of pressure is achieved (Use the 30° side lying position in preference to 90°) (See Appendix 7)
 - Avoid positioning patients on areas of erythema, if practicable
 - Reposition the individual using manual handling techniques and equipment that reduce friction and shear e.g. slide sheets, slide board, turn assist, HoverMatt.
 - Promote sitting out of bed in an appropriate chair with a specialist cushion for limited periods of time. Ensure that the patient is positioned correctly e.g. postural alignment and support, even weight distribution, balance and stability using functions such as tilt in space, back angle recline, adjustable seat dimensions, lateral supports, leg rests.
 - If unable to sit out of bed, utilise profiling function on beds or specialist beds with a chair function e.g. VersaCare bed.
 - If clinically appropriate reposition unstable critically ill individuals who can be repositioned using slow, gradual turns to allow time for stabilization of hemodynamic and oxygenation status.
 - If the patient is too clinically unstable initiate frequent small shifts in body position every hour, and/or wedges/pillows to off-load sacrum/coccyx to maintain a regular repositioning schedule and to supplement regular repositioning. Also consider continuous lateral rotation therapy with specialist beds – contact the Hospital Equipment Service.
 - Consider patients bed positioning regime: supine/ prone/ axillar (e.g. in prone check toes, dorsum of foot, genitalia, sternum, chin, forehead and cheekbones) (See Appendix 9 Prone Positioning)
 - Implement an early mobilisation program that increases activity and mobility as soon as clinically indicated

Heel areas

- Use heel off-loading devices e.g. heel elevators/wedges/pillows when lower leg / foot assessment indicates that a patient is at risk of developing a pressure injury or has a pressure injury. Offload the heel completely in such a way as to distribute the weight of the leg along the calf without placing pressure on the Achilles tendon or popliteal vein.-Use a slide sheet when moving the patient up the bed.

Plantar and forefoot

- Check that the patient’s foot is not pressed up against the end of the bed as this may become a high-pressure area if not identified – patients with increased comorbidities are at risk of pressure area on the forefoot or apices of toes.
- Consider adding in a bed extension on the bed if the patient is too tall for the bed or use the knee break to prevent the patient slipping down the bed.

Medical Device Related Pressure Injuries

- Medical device related pressure injuries can be prevented by understanding the risk factors and adherence with the actions below:
 - Use of correct fitting /size device
 - Ensure the device is sufficiently secure without creating additional pressure on the skin
 - Keep skin clean and dry under the medical device
 - Regular repositioning and rotating of the device (e.g. Endotracheal Tubes)
 - Use of preventative dressings (e.g. thin multilayered silicone absorbent dressings)
 - Regular skin inspection under and around the device, including under any preventative dressings, at least twice daily. More frequent inspection if the skin is oedematous or fragile.
 - Ensure correct fitting and checking of devices/orthoses/anti-embolic stockings, Darco shoes, CAM boots and casts.

Support Surfaces / Devices

- Select a support surface that meets the individual's need for pressure redistribution based on the following factors¹:
 - Level of immobility and inactivity
 - Need to influence microclimate control and shear reduction
 - Size and weight of the individual
 - Number, severity and location of existing pressure injuries
 - Risk for developing new pressure injuries and/or deterioration of existing pressure injuries.
- Pressure redistribution surfaces/devices (PRD) can be accessed via the Hospital Equipment Service (HES) at SCGH and ward storage area/ Equipment Storage Room at OPH. (See Appendix 6 to access consignment PRD)
- All patients using pressure redistribution devices require a minimum of second daily pressure injury risk assessment (see guidelines Appendix 1).
- Refer to support surface guide for assistance with choosing correct support surface to redistribute pressure.
- Patients on redistribution surfaces/devices that require assistance should be repositioned a minimum of two hourly. When repositioning check for any change in skin condition or deterioration and alter pressure injury management and wound care plans accordingly. Document the change in skin condition in the medical notes.
- Pressure redistribution surfaces should be checked each shift to ensure correct settings.
- If equipment is no longer required, it should be cleaned prior to returning to the HES at SCGH and ward storage area/Equipment Storage Room at OPH. Any faults observed with the equipment should be reported to:
 - SCGH, notify the HES apply a faulty tag to the equipment and arrange equipment to be exchanged.
 - At OPH, inform Ward Clerk to complete an EMPAC.
- SCGH only: For patients with or at risk of pressure injury being transported from SCGH to rural destinations consider using the ambulance/transport REPOSE trolley mattress overlay. Sourced from HES through Discharge Co-Ordinators at SCGH.

The following should not be used as Pressure Redistribution Devices: sheepskin, donut shaped cushion/gel, water filled gloves.¹

Intensive Care Unit (ICU)

All risk assessment documentation, pressure prevention and management plans are completed on the ICU flowchart and care plan.

As ICU patients are considered 'Very High Risk', all patients are to:

- be placed on a pressure redistribution device
- have prophylactic dressings in place
- be reviewed by a dietitian
- be repositioned and receive pressure area care frequently

Once transferred from ICU, re-screening and appropriate paper documentation will be commenced by ward staff.

Perioperative Area

Pre-operatively the operating theatre nurse must complete a comprehensive physical assessment of the skin and pressure areas and record this in Theatre Management System (TMS).

To prevent the development of pressure areas, staff will:

- Apply prophylactic dressing as required.
- Consider positioning required for the surgical procedure such as:
 - the anatomical complications of the specific position and/or surgery being undertaken
 - the physical limitations of the patient.
- Utilise appropriate pressure relieving medical devices to facilitate positioning the patient securely and safely for the duration of the surgical procedure with the use of e.g. gel pads, gel mats and padded positioning equipment.

At the end of the surgery the operating theatre nurse must complete a comprehensive physical reassessment of the skin and pressure areas and record this in TMS.

- **SCGH** all patients post-operatively must have a Waterlow Risk Assessment sticker completed and inserted to the relevant medical notes to record the skin and pressure area status for the ongoing healthcare provider. This sticker is placed in the patient's notes (day cases on the reverse of the Day Procedure Checklist [MR710], for inpatients in the inpatient medical record. The sticker states the length of time the patient was on the theatre table and if a prophylactic dressing had been utilised (see Appendix 8).
- **OPH** the Waterlow Risk assessment is completed in PACU prior to transfer to ward area

All pressure injuries and/or skin tears must be documented in the inpatient notes utilising the relevant Pressure Injury Alert sticker or the Skin Tear Alert sticker (see appendix 5).

3.0 Presence of a Pressure Injury

All pressure injuries, either pre-existing or acquired during the current admission, must:

- Be documented in the patient's medical record using the Pressure Injury Alert sticker (see Appendix 4), using one sticker per pressure injury. Documentation in the patient's notes should include:
 - the stage of the pressure injury as per appendix 2: Stage I, II, III, IV, unstageable pressure injury: depth unknown, suspected deep tissue injury: depth unknown.

- wound dimensions (mucosal membrane pressure injuries should be managed and documented but must not be staged)
- Have a Wound Management plan commenced.

Assess the patient's level of pain and liaise with MO to ensure optimal management.

Contact the Wound Management Service for Pressure Injury Stage II, III, IV, Unstageable or Suspected Deep Tissue Injury.

Refer to Occupational Therapy for offloading equipment, seating assessment.

Consider Audio Visual Production Unit Medical photography or Silhouette system to assist with monitoring wound changes.

Document in the patient medical record if a patient with a known pressure injury refuses recommended interventions.

A completed Pressure Injury Alert sticker is essential for correct coding for the patient's admission

4.0 Patient / Carer Education

Wound care plans should be developed in partnership with patients and carers.

Staff will:

- Educate patients and / or carer on the causes, prevention and treatment of pressure injuries (where relevant) and document in care plan or medical notes.
- Provide written support material or alternative education through hospital TV channels (e.g. *Patient Safety-First Leaflet and TV channel, Pressure Injury: What you should know, Patient information TV channels at SCGH/OPH*)

Patients, families and/ or carers should be considered as active participants in the prevention and management plan. They should receive information about the relevant risk factors and how to prevent pressure injuries.

Patients need to be encouraged to self-reposition, maintain mobility and activity where possible.

5.0 Documentation on Transfer/Discharge

Prior to discharge, Nursing/Allied Health staff will:

- Complete an assessment and management plan for all identified 'at risk' or existing pressure injury patients.

Prior to transfer, Nursing/Allied Health staff will:

- Document a handover and management plan of pressure injury.
- Communicate with the relevant care facilities/ providers/ carers.

Medical staff will include pressure injuries in the patient discharge summary.

A patient discharged from the hospital with an existing pressure injury, requires a written assessment and management plan to be sent to their primary health provider and a copy provided to the patient

Compliance, Monitoring and Evaluation

Ongoing monitoring and reporting of pressure injuries will go through the Datix Clinical Incident Management System (Datix CIMS), in line with the CIM Policy (MP 0122/19).

Compliance with this policy is overseen by the Wound Management Head of Department and the Multidisciplinary Wound Management Working Group.

Governance and compliance reporting is provided to the Comprehensive Care Committee, Clinical Governance Committee and Nursing Executive Committee.

See: NSQHSS Audit Schedule on Safety Quality & Performance HealthPoint Page

Related Documents / Resources

- [NMHS Clinical Incident Management Policy](#)
- [The National Pressure Ulcer Advisory Panel \(NPUAP\)](#)

References

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3. Santamaria N, Gerdzt M, Sage S, McCann J, Freeman A, Vassiliou T, et al. A randomised controlled trial of the effectiveness of soft silicone multi-layered foam dressings in the prevention of sacral and heel pressure ulcers in trauma and critically ill patients: the border trial. International Wound Journal. 2015;12(3):302-308. Level 2 Evidence
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Applicable Standards:				

Appendix 1: Guidelines for use of Support Surfaces

NOT At Risk	Bed Device	Chair Device
No pressure injury present	- Standard hospital foam mattress. - Range 247kg-254kg - Bariatric foam (0-450kg) - Tempur-Med (memory foam) - Range 20kg-150kg - Compella (113-454kg)	
At Risk	Bed Device	Chair Device
Pressure injury stage 1-2	- Tempur-Med (memory foam) Range 20kg-150kg <u>Alternating pressure mattresses</u> - Talley Quattro Acute (0-250kg) - Talley Quattro Plus (0-200kg) - Talley Quattro Overlay (0-200kg) - Alpha Response Overlay (40-135kg) - Alpha Response Mattress (40-160kg) or <u>Constant low pressure/air loss mattresses</u> - VersaCare (30-227kg) - Progressa (113-455kg) - Compella (113-454kg)	- Foam cushions - Basic – 100kg - ApolloHealthcare - 152kg - Dyna-Tek Owl - 152kg - Roho cushion Size S, M, L, XL - Gel Tru-Life – 130kg Refer to Wound Management or OT for custom made cushion requirements
High to Very High Risk	Bed Device	Chair Device
Pressure injury stage 2 and above; and/or multiple pressure injuries. Inability to keep patient positioned off pressure injury	<u>Alternating pressure mattresses</u> - Quattro Acute (0-250kg) or <u>Constant low pressure/air loss mattresses</u> - VersaCare (30-227kg) - Talley Quattro Acute (0-250kg) - Talley Quattro Plus (0-200kg) - Compella (113-454kg) - Progressa (113-455kg)	- Foam cushions - Basic – 100kg - ApolloHealthcare - 152kg - Dyna-Tek Owl - 152kg - Roho cushion Size S, M, L, XL - Gel Tru-Life – 130kg Refer to Wound Management or OT for custom made cushion requirements

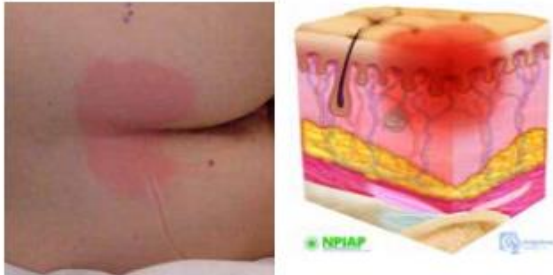
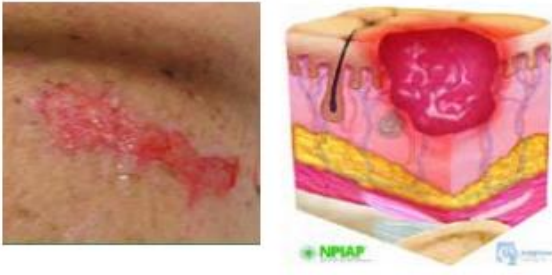
A complete patient assessment (i.e. weight, body shape, Waterlow Risk Assessment Tool, pressure injury staging, mobility) is required before selecting the bariatric integrated low air loss /constant low-pressure mattress (Compella) with turning function. Bariatric foam (450kg) is available from the Hospital Equipment Service (HES) if the Compella bed is not required.

Patients with spinal injury or multiple fractures - DO NOT use pressure redistribution support surfaces without medical approval.

Contact the Wound Management Service, HES or After-Hours CNS if further advice is required eg low weight/cachectic or bariatric patients.

Appendix 2: Staging of Pressure Injuries

Table 12.2: Pressure injury Category/Stages: photographs and illustrations
(see the guideline Introduction for credits)

Category/Stage I Pressure Injury 	Category/Stage II Pressure Injury 
Category/Stage III Pressure Injury 	Category/Stage IV Pressure Injury 
Unstageable Pressure Injury (covered in eschar or slough) 	Suspected Deep Tissue Injury 

Appendix 3: Waterlow Risk Assessment Tool

Waterlow Risk Assessment Tool (More than one score per category can be used)			Date												
			Time												
Build / Weight	Average	0													
	Above average	1													
	Obese	2													
	Below average	3													
Skin Type	Healthy	0													
	Tissue paper	1													
	Dry	1													
	Oedematous	1													
	Clammy, pyrexia	1													
	PI grade 1	2													
	PI grade ≥ 2, SDTI or unstageable	3													
Gender	Male	1													
	Female	2													
Age	14 - 49 years	1													
	50 - 64 years	2													
	65 - 74 years	3													
	75 - 80 years	4													
	81+ years	5													
Continence	Complete / catheterised	0													
	Urinary incontinence	1													
	Faecal incontinence	2													
	Urinary + faecal incontinence	3													
Mobility	Fully	0													
	Restless / fidgety	1													
	Apathetic	2													
	Restricted	3													
	Bedbound	4													
	Chairbound	5													
Tissue Malnutrition	Terminal cachexia	8													
	Multiple organ failure	8													
	Single organ failure	5													
	Peripheral vascular disease	5													
	Anaemia (Hb <80g/l)	2													
	Smoking	1													
Neurological Deficit	Diabetes, multiple sclerosis, stroke	6													
	Motor / sensory	6													
	Paraplegia	6													
Major Surgery or Trauma	Orthopaedic / spinal	5													
	On table > 2hrs (last 48hrs)	5													
	On table > 8hrs (last 48hrs)	8													
Medication (Max of 4)	Cytotoxics	1													
	Long term steroids	1													
	High dose steroids	1													
	Anti-inflammatory	1													
Malnutrition (MST)	Has patient lost weight recently?														
	No	0													
	Unsure	2													
	Yes: 1 - 5 kg	1													
	Yes: >5 - 10 kg	2													
	Yes: >10 - 15 kg	3													
	Yes: >15 kg	4													
	Yes: Unsure how much	2													
	Is patient eating poorly / lack of appetite?														
	No	0													
Yes	1														
Total Score															
Initials															

Appendix 4: Pressure Injury Alert Sticker

Pressure Injury Alert



Was the pressure injury present on admission? Yes No

Anatomical location of pressure injury: _____

Circle the pressure injury classification stage:

Suspected

I, II, III, IV, Unstageable, Deep Tissue

Has a wound management plan been commenced? Yes No

Has a pressure management plan been commenced? Yes No

Has a Datix CIMS been notified? Yes No

CIMS Ref Number: _____

Name, designation & signature **Date:** _____ **Time:** _____

NB. This sticker is NOT to be used for Mucosal Pressure Injuries

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Please use one sticker per pressure injury

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Appendix 5: Pressure Injury Risk Magnet used on Journey Board



Appendix 6: Flowchart on activating a consignment mattress at OPH

ACTIVATING A CONSIGNMENT (RENTAL) TALLEY QUATTRO MATTRESS

Nursing staff to assess the patient's risk of pressure injury and establish the need for Talley Quattro air mattress

If non available from OPH stock, request PSA/nursing staff to get mattress from Equipment Storage Room (E Block, Level 1). Provide them with patient sticker or patient name

PSA/ nursing staff collects the requested Talley Quattro Plus mattress, pump and red plastic bag from the storage area and **completes the "Consignment Log Sheet"** kept with the mattresses. It is important to place the patient sticker/write patient's name on the log sheet in the relevant space.



PSA/staff transports the mattress and the pump to the ward and set it up on the bed.

Mattress is to be placed directly onto a bed frame, **not on top of any existing mattress**. Allow 10-15 minutes for mattress to inflate before placing the patient on the mattress. The pump will display message "ACTIVE MEDIUM" when the setup process is complete

If there are any issues or concerns with the air mattress, **return the mattress and the pump** to the storage area with the note of what the issue is and take a new mattress. **Complete the "Consignment Log Sheet"** again for new mattress.

Area Manager to activate the rental with HospEquip on the next business day

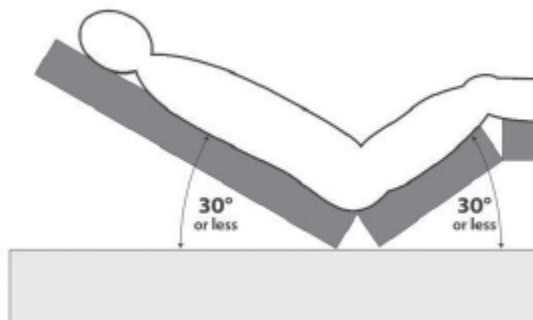
When no longer required place the mattress in red plastic bag and return to the Equipment Storage Room. Fill in Return Date on "Consignment Log Sheet".

Appendix 7: Positions to reduce pressure

30 side lying position



Head of bed elevation.



Appendix 8: Theatre Post-Operative Waterlow Risk Assessment sticker

Theatre Post-Operative Waterlow Risk Assessment

Major Procedure	5		Prophylactic Dressing Applied
High Risk > 6hrs	8		Yes ☐ N/A ☐
Med. Risk > 2hrs	5		Site(s):
Low Risk < 2hrs	0		

Signature: _____ **Designation:** _____

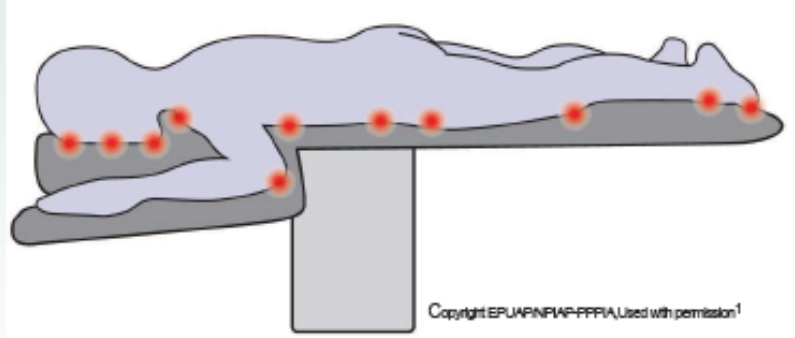
Name (printed): _____ **Time:** _____ **Date:** _____

Appendix 9. Pressure Injury Prevention-PIP Tips for Prone Positioning

- General Tips:**
- Use a pressure redistribution surface (for those not on a bed specifically designed for proning)
 - Follow manufacturer instructions when using beds, positioning devices, prophylactic dressings and other products.
 - Positioning devices/pillows are needed to offload pressure points.
 - Involve enough trained staff to avoid friction-shear when repositioning. May reposition into swimmer position.
 - Microshifts and small position changes should be performed while prone, especially in non-rotating beds.
 - Assess all pressure points :
 - Prior to proning (anterior surfaces). Prior to returning to supine position (posterior surfaces).
 - When alternating arm position in swimming arm position, assess integrity of skin of arm/head/face.
 - Document all skin assessments and preventive measures.

Pressure Points:

- Forehead
- Chin
- Cheeks
- Nose
- Clavicle - shoulder
- Elbow
- Chest - breasts
- Genitalia - penis
- Anterior pelvic bones (iliac crests, ischium, symphysis pubis)
- Knees - patella
- Dorsal feet and toes¹
- Under/around medical devices



Pay special attention to the following areas:



HEAD

- Apply soft silicone multi-layered foam prophylactic dressings to pressure points on face.
- Manage moisture: Suction oral secretions. Use liquid skin protectants/sealants on face. Change foam dressing pm. Apply hydrofiber/calcium alginate dressing under prophylactic dressing to manage excess moisture.
- Apply thin foam dressings under medical devices. Avoid multiple layers of dressings that increase pressure.
- Offload head with offloading device(s): Consider the density of foam, height of the cushion, angle of the face, and endotracheal tube (ETT) positioning when selecting an appropriate device.
- With manual proning, shift patient's head q 2 hours; re-position head q 4 hours. May adjust timing to patient needs.
- Note: commercially available ETT securement devices may contribute to increased skin breakdown in prone patients. Assess skin carefully. Consider tape to secure ETT during proning.
- Maintain eye care to prevent corneal abrasions. Apply ophthalmic lubricant. Tape eyelids shut horizontally.
- Ensure tongue is inside patient's mouth. A small soft bite block may help. Assess tongue for injury.



TORSO

- Place EKG leads on back while proning.
- Apply prophylactic foam dressings to pressure points.
- Ensure central lines, arterial lines and cannulas are secured (e.g., sutured).
- Empty ileostomy/colostomy pouches and pad around stoma site.
- If receiving enteral feedings, turn off feeding 1 hour before prone position turn. Resume once in prone position as ordered.
- Secure all tubes and devices away from skin; protect surrounding skin with prophylactic dressings and bridge areas with positioning devices.
- Create channels for tubes with positioning aids. Ensure that there are no unsecured devices under the torso.



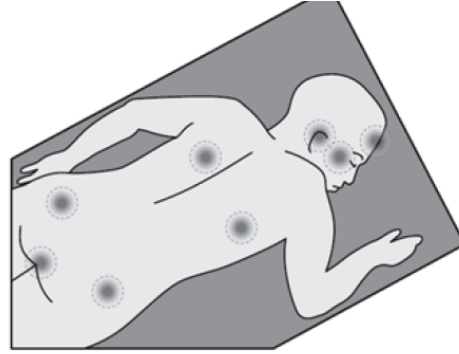
LEGS

- Apply prophylactic foam dressings to pressure points (e.g., patella and pretibial area).
- Remove securement devices and align urinary catheter/fecal management device toward foot of bed.
- Ensure that there are no unsecured devices under legs. Offload feet.

BREASTS & GENITALIA are particularly sensitive tissues that should be offloaded and protected.

Swimming/Freestyle Position

- Patients in standard prone position can be repositioned using the swimming position.
- Alternate the position of the arms and direction of the head, in a manner that is similar to that of a freestyle swimmer.
- Pressure points on the diagram correspond to the body surface facing the mattress.



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